



Bipolar Disorder

Bipolar disorder is a chronic mental disorder characterized by episodes of mania or hypomania and depression, affecting a person's mood, energy level, activity, and thoughts. It is reported to affect approximately 37–40 million people worldwide. The disorder generally begins in late adolescence or early adulthood and may follow a lifelong recurrent course. Bipolar disorder can lead not only to changes in mood but also to significant impairments in work, education, family relationships, and daily functioning. It is also associated with an increased risk of suicide, anxiety, and substance use disorders.

During a manic episode in bipolar disorder, excessive elation or irritability, a marked increase in energy and activity, decreased need for sleep, rapid speech and racing thoughts, distractibility, an exaggerated sense of importance or power, and increased risk-taking behaviors may occur. In severe cases, psychotic symptoms involving a loss of contact with reality may emerge. During a depressive episode, a depressed mood, loss of interest and pleasure, decreased energy, difficulty concentrating, changes in sleep and appetite, feelings of guilt or worthlessness, hopelessness, and thoughts of death or suicide may be observed. The persistence of depressive symptoms for at least two weeks, during most of the day and nearly every day, is important in distinguishing them from ordinary mood fluctuations. Hypomania is an episode similar to mania but less severe and generally does not impair functioning to the same extent.

There are two main types of bipolar disorder: Bipolar I and Bipolar II. Bipolar I disorder is characterized by distinct manic episodes, and depressive episodes may also accompany them. Bipolar II disorder is characterized by a pattern of hypomanic and depressive episodes, with no history of a manic episode. The NIMH also defines cyclothymic disorder (a long-term pattern of recurring mild hypomanic and depressive symptoms), characterized by recurrent hypomanic and depressive symptoms that are milder and shorter in duration, as a separate type of bipolar disorder.

The exact cause of the disorder is not fully known. Research indicates that, in addition to genetic predisposition, biological, psychological, and social factors play a role together. No single gene causes the disorder; the interaction between the effects of multiple genes and stressful or traumatic life events may be important. Bereavement, relationship loss, and other sources of stress may trigger or worsen symptoms.

Treatment is possible for bipolar disorder, and recovery can be pursued. While mood stabilizers and appropriate antipsychotic medications form the basis of treatment, psychotherapy and psychosocial interventions are important complementary components of treatment. Some mood

stabilizers, such as lithium, may help prevent episodes and reduce the risk of suicide. Psychological approaches such as cognitive behavioral therapy, psychoeducation, and interpersonal/social rhythm therapy may contribute to reducing symptoms and relapses. Regular sleep, physical activity, healthy nutrition, stress reduction, and monitoring of mood also support treatment.

In conclusion, bipolar disorder is a serious illness that is not merely characterized by mood changes but can affect a person's functioning, quality of life, and social relationships, yet it can be managed with appropriate and continuous treatment. Early and accurate diagnosis, individualized treatment, psychosocial support, and continuity of treatment are fundamental elements in controlling the disorder.

References:

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